



A SOOTHEMADE BOOK

The First Six Weeks

A newborn survival book, week by week.

Soothemade

A SMALL APOTHECARY · MADE SLOWLY

Dedication

For the parent in the kitchen at 3am, crying because they cannot remember whether the last feed was at 1am or 11pm.

You are doing it. Even when you cannot remember.

A note from Maya

I am not a pediatrician. I am not a postpartum doula. I am a parent who, with my first, brought home a baby and stood in the kitchen at 3am crying because I could not remember whether I had fed him at 1am or 11pm.

The hospital sent us home with a folder of glossy pamphlets, none of which addressed the actual question I had, which was "is this normal." A nurse handed me a piece of paper that said "Sleep when the baby sleeps." I had already been awake for thirty hours. I wanted to set the paper on fire.

This book is the thing I wish I had on the kitchen counter. Not a methodology. Not a parenting philosophy. A scaffolding for the kind of week where you cannot keep track of anything.

It assumes you are doing your best. It assumes some days will be worse than the day before. It assumes you will sometimes skip a chapter. All of that is allowed.

With care, Maya

None of this is medical advice. If something feels wrong with your baby or with you, call your provider. If you are in crisis, do not look it up here. Call 988 (US Suicide and Crisis Lifeline) or the US Maternal Mental Health Hotline at 1-833-852-6262.

Chapter 1: The One Thing to Remember

If you remember nothing else from this book.

The first six weeks are not a smaller version of parenting later. They are a separate event.

It is not "the start of motherhood, but harder." It is its own time. Survival mode is the correct mode. The "real you" is not coming back this week. The version of you who is here now is enough.

Six weeks is also not a magic number. You will not wake up on day forty-three and be sleeping through the night. But something does shift, somewhere in there, almost universally. Hold on to that.

The chapters that follow walk through the six weeks. They are short on purpose. Read the chapter for the week you are in. Skip the rest. Come back to it next time.

Chapter 2: The First 48 Hours

The first two days, hospital and homecoming, run on their own clock. Here is what to actually do.

Things to do, no asking.

Drink water every time the baby feeds. Set a glass in your reach. Refill it without ceremony. Take photos. The first feed. The first diaper. You holding the baby. The partner holding the baby. You do not have to post any of them. They are for you.

Write down what the nurses tell you. Discharge instructions get fuzzy fast. If you can, do not put yourself in charge of any household decision yet. Not the laundry. Not the meal plan. Not the thank-you cards. Sleep when you can. Not when the baby sleeps. *When you can*. Sometimes that is at 4pm.

Things you'll be tempted to do, don't.

Don't send a long birth-story text to everyone you've ever met. You can do that in week three. Send a single photo and the baby's name. That is enough. Don't make a list of thank-yous. The list will keep. Don't try to shower in a way that feels normal. You are not normal. The shower can be three minutes. The shower can be a wet washcloth. Both count. Don't Google "is my baby breathing normally" at 2am. Watch the baby. If you are unsure, call the nurse line. Do not crowdsource.

A note on tears. If you cry on day three, that is the baby blues. About 80% of birthing parents have a wave of intense emotion between day three and day five. It does not mean you are not bonding. It does not mean you have postpartum depression yet. It means your hormones are doing a forty-foot drop in seventy-two hours. If the tears are still daily by week three, that is a thing to flag. There is a chapter on this at the end.

Chapter 3: Week 1, Survival Mode

The first week home is mostly logistics and shock. You are tracking feeds, diapers, sleep, and not much else.

What is normal:

- Eating: roughly every 2 to 3 hours, sometimes more often. Newborn stomachs are tiny.
- Diapers: 6+ wet, 3+ dirty by the end of the first week. Color changes (meconium to yellow) over the first few days.
- Sleep: 14 to 17 hours per 24, in stretches of 1 to 3 hours.
- Weight: the baby loses a little after birth (up to 10%) and starts gaining back by week one to two. The pediatrician tracks this at the first visits.

What is not normal:

- A fever (any temperature elevation in a newborn) warrants a call.
- Very few or no wet diapers.
- Yellowing of the skin or eyes that is getting worse.
- Trouble breathing, fast or noisy breathing.
- Hard to wake, unusually limp.
- Inconsolable crying for an extended stretch.

Call the pediatrician. The nurse line is open for these calls. They are not interruptions. They are the job.

The parent. Eat three times a day. Drink water at every feed. Sleep when you can. Shower every day or two. Bleeding postpartum is real and lasts weeks. Stock the supplies. Mesh underwear is a gift. Pain reliever as your provider recommended. Stairs less than you think. Help where you can get it.

The line. The nurses go home with you. They are gone. You are the team now. Whatever the discharge papers said, follow that. Bring the questions to the first pediatrician visit.

Chapter 4: Week 2, The Reality Settles In

This is when the adrenaline starts to wear off. You may be crying more, not less. You may be quieter. You may ask the partner if "this is forever."

The mental health watch. Around day 10 to 14, the baby blues should be easing. If you are crying more, not less. Saying "I don't feel anything for the baby." Saying "everyone would be better off without me." Not sleeping when the baby sleeps because you can't stop checking. Talking about scary intrusive thoughts you can't get rid of.

This might be postpartum depression or postpartum anxiety. It's common, about 1 in 7 birthing parents. It is treatable. Call the provider. If you cannot, ask your partner to call. If something is bigger than a provider call, the US Maternal Mental Health Hotline is 1-833-852-6262, 24/7, free, confidential.

The body. Two weeks postpartum, you should be feeling slightly less destroyed. Slightly. Bleeding may be reducing. You may have more bandwidth for a meal. If anything is worse, not better, call.

The baby. They are starting to be slightly more awake during the day. The eye contact is forming. You will be in the chair, looking at them, and they will look back. Sit with that. It does not have to be a milestone. It can be a Tuesday at 2pm.

The first pediatrician visit. Around now. Bring the questions you have been writing down. Ask the ones you are most afraid to ask. The pediatrician has heard all of them. The visit is the place for them.

Chapter 5: Week 3, The "I Should Be Fine By Now" Trap

Around week three, the casseroles stop, the texts taper off, and you start thinking you should be "back to normal."

You are not back to normal. Your hormones just took the second drop. Your milk supply (if breastfeeding) is still regulating. You may be sleeping less, not more, because the baby's sleep is changing.

This week is when most parents check out of "intense" mode and assume things are stable. Don't.

What to keep doing:

- Asking for help (even if the offers have stopped, you can ask).
- Stocking snacks within reach.
- Sleep when you can.
- Walking outside, briefly, at least once a day if you can.

What to start. A short routine, very loose. Not a schedule. A loose rhythm. Mornings tend to be the time of day the baby is most settled. Afternoons tend to be harder. Evenings tend to be the witching hour. The pattern is not yours. The pattern is the baby's. You are the witness.

A note on visitors. By week three, some people will assume you are "open." You can still say no. Use the scripts from another book if you have it (*30 Scripts for the Postpartum Visitor* by Soothemade). The boundary is yours.

Chapter 6: Week 4, The Stretch

Week four is the longest week.

The newness is gone. The novelty for visitors has faded. The partner may be back at work or back to old commitments. The casseroles definitely stopped. And you are still in survival mode, but it feels like you are supposed to be out of it.

You are not. The first six weeks is the first six weeks. Week four is the middle. The middle is the long stretch.

What helps:

- One outing per day, even brief. A walk around the block. The mailbox. The grocery for one item.
- One conversation per day with an adult who is not your partner. A text counts. A voice memo counts. A 5-minute call counts.
- One thing per day that is just yours. A cup of tea sitting down. A song you used to love. A book you can read one page of.

What does not help:

- Comparing yourself to anyone on social media.
- Reading about "the right way" to do anything.
- Setting weekly goals.
- "Should" thoughts of any kind.

The honest check. Are you eating? Are you hydrated? Have you slept any 4-hour stretches recently? Are you crying daily? Are the intrusive thoughts increasing? Is your mood worse than last week, or about the same?

If any of these are concerning, the provider is open. The line is open.

Chapter 7: Week 5, The Slight Lift

For most parents, somewhere around week 5, something lifts.

It is not dramatic. You do not wake up on day 35 and feel like yourself. But you may notice that the morning was easier than last week. That the baby cried less. That you ate sitting down. That you laughed at something a friend said.

These are small signals. They are not promises. The next week may be hard again. The lift is not linear.

What is happening:

- The baby is starting to have longer awake stretches and slightly longer sleep stretches. The 4-hour-at-night stretch is closer.
- Your hormones are continuing to shift. Some symptoms are easing (others may be appearing).
- You are getting better at reading the baby. Cries are starting to differentiate.

What to do with the slight lift:

- Not too much. Resist the temptation to "catch up" on all the things. The catching up is the trap.
- Pick one thing. Reply to one specific friend. Schedule one specific outing for next week. Make one specific phone call you've been putting off.
- Keep doing the small things. The basics that got you here. Water. Food. Sleep. One outside walk. One adult conversation.

Chapter 8: Week 6, The Appointment

Around week six, the postpartum appointment.

This is your appointment, not the baby's. The provider checks your physical recovery. They ask about your mood. They ask about your sleep. They ask about your relationship and your support system. Some of the questions are awkward. Answer them honestly.

Things to bring to the appointment:

- Any questions you have written down.
- Information about how you are eating, sleeping, feeling.
- Honest information about your mental health. The provider can help. If you are not okay, this is the appointment to say so.
- Information about anything physical that doesn't feel right. Lingered pain. Heavy bleeding. Anything else.

Things to expect:

- A physical exam.
- A mental-health screening (often a brief questionnaire).
- A conversation about return to activity, including sex and exercise.
- A conversation about contraception.
- A scheduled follow-up if needed.

The myth of "you are healed." The 6-week appointment is a milestone, not a finish line. Most providers are clearing you for activity, not declaring you back to normal. Recovery continues for months. Some things (pelvic floor, mood, sleep) continue to evolve well beyond 6 weeks.

A small principle for after the appointment: take the clearance as permission, not as an instruction. If you are not ready for exercise, sex, work, or anything else that was "cleared," that is also fine.

Chapter 9: Mental Health, Always

This chapter is at the back so you can find it.

Postpartum mental health is real. Common. Treatable. The treatment is good. The sooner you start, the faster you get yourself back.

What to watch for:

- Crying daily, more than once.
- Intrusive thoughts about harm to yourself or the baby.
- Persistent sadness or numbness.
- Inability to sleep even when the baby sleeps.
- Inability to eat.
- Rage that scares you.
- Anxiety that runs the day.
- Loss of pleasure in things that used to bring joy.
- Withdrawal from people you love.
- Thoughts of "everyone would be better off."

If two or more of these are true, call a provider. A perinatal mental-health specialist is best.

Crisis lines.

- US, 988, Suicide and Crisis Lifeline (call or text).
- US, 1-833-852-6262, Postpartum Support International HelpLine.
- US, 1-833-943-5746, Maternal Mental Health Hotline (24/7).
- UK, 116 123, Samaritans.
- Canada, 9-8-8, Suicide Crisis Helpline.
- Australia, 1300 726 306, PANDA.

A reminder: asking for help is not a sign that something is wrong with you. It is a sign that you are paying attention.

Chapter 10: A Final Note

You got through six weeks.

That sentence will not feel impressive at the time. It will feel like you barely held the days together. You did not have a system. You did not have a routine. You did not "thrive."

But six weeks have passed. The baby is here. You are here. The relationships that held during these weeks are stronger for it. The ones that did not are different now, and you have information.

Whatever comes next, the first six weeks were a foundation. You laid it. On no sleep. With a body that was healing. With your hands full of someone else's life.

That is the work.

With care, Maya

About Soothemade

Soothemade is a small apothecary of printables, planners, cards, and books for new parents.

The printable companion to this book is the **Newborn Survival Planner** at notes.soothemade.com, with daily logs and a "what we tried" page.

Also from Soothemade

- **How to Read Your Baby**, for the cues the chapters reference.
- **30 Scripts for the Postpartum Visitor**, for the doorbell during the six weeks.
- **The Partner's Postpartum Playbook**, for the other adult in the house.
- **The Asking for Help Script Pack**, for the village you may need to build.

This book is for planning and personal use only. It is not medical advice.

A Soothemade book. Made slowly, in plain language.